Track Responsibility: Northumbria regional map (as is)

Place-based view of how referrals move, where responsibility changes hands, how staff work around the gaps, and what happens when those workarounds fail.

Evidence: Northumbria site visit, 2 to 4 November 2026. Five interviews (P01 referral ops, P02/03 RTT and funding, P04 surgical booking lead, P05 cancer and 2WW booking), one gastro consultant session (C1) and two session briefs (S1 outside e-RS and RTT, S2 surgical admin). Emerging findings, not yet validated.

Stages

Into the trust

1. Need identified and route chosen

Responsibility overview

CLEAR

Clinical: referring clinician (GP, consultant or A&E).
Admin: referring secretary.
The decision sits with the referrer.

Patient — Patient / carer

Told they need another service, often after A&E or an outpatient visit.
S1

Referring clinician — GP, consultant or A&E

Decides the patient needs another service. May not know a related referral already exists.
S1

WORKAROUND
Check OneView or ask colleagues whether a related referral exists.

FAILURE POINT
No warning that a related referral or pathway exists.

CONSEQUENCE
Duplicate pathways, inflated PTL, a wasted or missed appointment.
Anecdotal – user · 2+ sessions

Referring admin — Secretaries, practice admin

Receiving clinician — Triager, consultant, nurse

Contact centre and booking — Referrals, appointments, RMS

RTT and funding team — RTT trackers, funding admin

2. Referral created and sent

Responsibility overview

SHARED / ASSUMED

Referrer assumes it has landed. Sending an email creates no status. A GP may tell the patient it was sent while it is still being prepared.

Patient — Patient / carer

Doesn't know if the referral has gone. The GP may say it has when it hasn't.
P02/03

Referring clinician — GP, consultant or A&E

Writes a free-text email or letter, sometimes covering several patients. Creates a new referral to chase or expedite.
P02/03

WORKAROUND
Raise a new referral to chase or expedite. Refer to several providers to "hedge bets".

FAILURE POINT
Free-text emails, sometimes covering several patients.

CONSEQUENCE
Incomplete information. Delay while it's chased by email.
Anecdotal – user · 2+ sessions

Referring admin — Secretaries, practice admin

Emails the receiving team. Several secretaries may send the same request.
S2

FAILURE POINT
Several secretaries send the same request.

CONSEQUENCE
Duplicates reconciled by hand on the receiving side.
Anecdotal – user · 1 session

Receiving clinician — Triager, consultant, nurse

Contact centre and booking — Referrals, appointments, RMS

RTT and funding team — RTT trackers, funding admin

3. Received and registered

Responsibility overview

GAP

Lands in a group mailbox or e-RS worklist. Nobody acknowledges back to the referrer. Re-keyed into PAS and saved to the X drive.

Patient — Patient / carer

Gets a portal message saying "you will be booked". Reads it as an appointment.
P01

FAILURE POINT
Message says "you will be booked" when nothing is booked.

CONSEQUENCE
Patient thinks it's an appointment and calls the contact centre.
Anecdotal – user · 1 session

Referring admin — Secretaries, practice admin

Gets chased by provider admin asking whether it has actually been sent.
P02/03

Receiving clinician — Triager, consultant, nurse

Contact centre and booking — Referrals, appointments, RMS

Internal referrals land in a group mailbox, routed by SOP. Uploaded to PAS and saved to the X drive.
P01, S1

WORKAROUND
Follow SOPs to find the right mailbox. Save every referral to the X drive.

FAILURE POINT
Group mailbox and manual saving.

CONSEQUENCE
Referrals have been lost before. Wrong or missing record. "One is too many."
Anecdotal – user · 2+ sessions

RTT and funding team — RTT trackers, funding admin

4. Triage and Advice & Guidance

Responsibility overview

SHARED / ASSUMED

Provider sees itself responsible once the request is in its inbox. e-RS status shows who replied last, not who must act next.

Patient — Patient / carer

Unaware triage is happening. A&G advice only reaches them through the GP.
C1

Referring clinician — GP, consultant or A&E

GP answers "more information" requests. Only the GP can close the A&G UBRN.
C1

Receiving clinician — Triager, consultant, nurse

Copies the NHS number into OneView, ICE, Great North Care Record and EndoWeb to rebuild the history. Then accepts, rejects, asks for info, leaves open or converts.
C1

WORKAROUND
Copy the NHS number across 4 to 5 systems.

WORKAROUND
Email other specialties behind the scenes rather than reject.

FAILURE POINT
Identifiers copied by hand between systems.

CONSEQUENCE
Wrong-patient risk.
Anecdotal – user · 1 session

FAILURE POINT
Re-triage to add information cancelled an appointment (cause unverified).

CONSEQUENCE
Patient lost their position.
Anecdotal – user · 1 session

Contact centre and booking — Referrals, appointments, RMS

2WW: logs on a local tracker, emails the coordinator, updates Somerset by hand.
S2, P05

WORKAROUND
Track 2WW on a local spreadsheet and update Somerset by hand.

FAILURE POINT
Same patient held in e-RS, spreadsheet, email, PAS and Somerset.

CONSEQUENCE
Later changes missed on urgent cancer pathways.
Anecdotal – user · 1 session

RTT and funding team — RTT trackers, funding admin

5. Booking

Responsibility overview

SHARED / ASSUMED

Booking staff believe the GP stays responsible until the first appointment (2 participants). Not confirmed Trust policy.

Patient — Patient / carer

Booked into the earliest slot, sometimes at the wrong site for them. Calls to change it.
P04

FAILURE POINT
Earliest slot chosen over the patient's preferred site.

CONSEQUENCE
Patient calls to rebook. Rework for booking staff.
Anecdotal – user · 1 session

Referring admin — Secretaries, practice admin

Receiving clinician — Triager, consultant, nurse

Reviews borderline referrals sent back by booking. Replies by email, which can take days.
S2

FAILURE POINT
Borderline cases sit in email for days.

CONSEQUENCE
Delay. The next person has to dig through old messages.
Anecdotal – user · 1 session

Contact centre and booking — Referrals, appointments, RMS

Reconciles the live e-RS worklist, a daily extract and PAS across three screens. Picks the clinic from codes held in memory and a SharePoint sheet. About 200 ASIs cleared in an afternoon.
P04, S2

WORKAROUND
Three screens. SharePoint sheet of clinic codes ("you hope it's up to date"). Daily extract emailed by team leads.

FAILURE POINT
Triage wording doesn't match clinic codes. Extract missing or stale.

CONSEQUENCE
Wrong clinic, delay, rework.
Anecdotal – user · 2+ sessions

RTT and funding team — RTT trackers, funding admin

Within the pathway

6. Waiting and RTT tracking

Responsibility overview

GAP

Provider holds deterioration risk while the patient waits, but the RTT team only starts tracking at week 17.

Patient — Patient / carer

Worklist re-adds trigger new notifications. Calls to ask what changed. Rarely knows their UBRN or real wait.
P01, S2

FAILURE POINT
Admin re-add sends a new notification with no real change.

CONSEQUENCE
Confusion and avoidable calls.
Anecdotal – user · 1 session

Referring clinician — GP, consultant or A&E

Re-refers, or the patient goes to A&E because of the wait (hernia example). Creates a second pathway.
P02/03

FAILURE POINT
Second referral for the same problem isn't linked to the first.

CONSEQUENCE
Two pathways. One appointment ends up as a missed appointment.
Anecdotal – user · 1 session

Referring admin — Secretaries, practice admin

Receiving clinician — Triager, consultant, nurse

Scans open A&G items after two or three weeks ("tail-gunning").
C1

WORKAROUND
Leave A&G open as a safety net. Protected 4-hour review slots, one owner.

FAILURE POINT
Status shows who replied last, not the next action.

CONSEQUENCE
Open items missed: A&G requests once went unactioned.
Anecdotal – user · 2+ sessions

Contact centre and booking — Referrals, appointments, RMS

Re-adds patients before the 180-day drop-off. Takes about six hours of calls on a Monday. Searches e-RS and PAS for patients with no UBRN.
P01, P05, S2

WORKAROUND
Re-add patients before the 180-day drop-off. "Hold patients in PAS". Local slot dashboard.

FAILURE POINT
Inbound calls compete with worklist work.

CONSEQUENCE
Booking slows. About six hours of calls on a Monday.
Anecdotal – user · 1 session

RTT and funding team — RTT trackers, funding admin

Starts tracking at week 17. Pieces together a Monday PAS download, CCS and notes in the RTT Tracker App. Asks the referrer if treatment is still needed.
P02/03

WORKAROUND
RTT Tracker App replaced 40+ spreadsheets with shared comments.

FAILURE POINT
Tracking only starts at week 17.

CONSEQUENCE
Stalled pathways and deterioration not seen earlier.
Anecdotal – user · 1 session

7. Seen, follow-up and internal onward

Responsibility overview

SHARED / ASSUMED

Moves to the hospital team after first attendance. Changes of destination arrive by email: "Can you change this?"

Patient — Patient / carer

Moved between clinics in complex specialties like urology. May repeat their story.
S2

Receiving clinician — Triager, consultant, nurse

Requests follow-ups, tests or other clinics by email or form. Coordinates other specialties behind the scenes.
C1, S2

FAILURE POINT
Follow-up email never arrives at booking.

CONSEQUENCE
Patient found months later with no follow-up booked.
Anecdotal – user · 1 session

Contact centre and booking — Referrals, appointments, RMS

Turns follow-up emails into PAS entries. Condenses duplicate requests.
S2

WORKAROUND
Check PAS and the inbox to spot duplicate follow-up requests.

RTT and funding team — RTT trackers, funding admin

Across boundaries

8. Onward to another trust or tertiary

Responsibility overview

GAP

Receiving trust is expected to act, but the originating trust cannot see what happens. Visibility drops sharply at the boundary.

Patient — Patient / carer

Little or no word when sent to another trust, e.g. the Freeman.
S1, S2

Referring clinician — GP, consultant or A&E

Consultant writes to another trust when treatment isn't available locally.
S1

Referring admin — Secretaries, practice admin

Sends the letter, then phones the receiving trust to check it landed.
S1

WORKAROUND
Phone the receiving trust to confirm the letter landed.

FAILURE POINT
Visibility break at the organisational boundary.

CONSEQUENCE
Nobody can say if the patient was seen. Originating trust reconstructs it later.
Anecdotal – user · 2+ sessions

Receiving clinician — Triager, consultant, nurse

Local MDT sends tertiary cancer referrals through Patient Pass.
C1

Contact centre and booking — Referrals, appointments, RMS

Tertiary hands care back by email. Staff link it to the local pathway in PAS.
S2

WORKAROUND
Re-link the tertiary handback to the local pathway in PAS.

FAILURE POINT
Handback arrives only as an email.

CONSEQUENCE
Return not linked. Local follow-up can be missed.
Anecdotal – user · 1 session

RTT and funding team — RTT trackers, funding admin

Checks if approval is needed, pulls evidence from OneView, enters it in Blueteq, saves it to the X drive. 5 to 10 minutes when clear.
P02/03, S1

WORKAROUND
Email clinicians for missing evidence.

FAILURE POINT
RTT clock can't stop during a funding decision.

CONSEQUENCE
Breach recorded. Patient waits up to four months. Appeal route unclear.
Anecdotal – user · 2+ sessions

9. Funding and prior approval

Responsibility overview

GAP

Commissioner or panel decides. The RTT clock keeps running and the appeal route is unclear.

Patient — Patient / carer

In limbo. A panel decision can take four months.
P02/03

Receiving clinician — Triager, consultant, nurse

Supplies evidence for funding when admin emails for it.
P02/03

RTT and funding team — RTT trackers, funding admin

Validates whether the pathway is open, stopped or closed. Reconciles duplicates by hand.
S1

WORKAROUND
Manual RTT validation to find out what actually happened.

FAILURE POINT
Duplicates only found at validation.

CONSEQUENCE
Inflated PTL and inaccurate reporting.
Anecdotal – user · 2+ sessions

10. Complete, close and inform

Responsibility overview

SHARED / ASSUMED

RTT team validates whether the pathway is open or closed. GP controls closure of A&G. Old UBRNs can stay live.

Patient — Patient / carer

Often learns the outcome through the GP, or by chasing.
C1, S2

FAILURE POINT
Changes only found when the patient calls.

CONSEQUENCE
Patient-led detection. Inaccurate reporting.
Anecdotal – user · 2+ sessions

Referring clinician — GP, consultant or A&E

GP closes the A&G episode and decides what to tell the patient.
C1

Receiving clinician — Triager, consultant, nurse

Contact centre and booking — Referrals, appointments, RMS

RTT and funding team — RTT trackers, funding admin

Line of responsibility handover

Analysis layers

Workarounds and failure points sit under each touchpoint above. These rows show handoffs, visibility, policy and clinical safety, and what this means for capabilities and platform teams.

Handoffs

What moves (information, responsibility) and how

HANDOFF 1 — Clinician to secretary. Decision and dictation move. Responsibility stays with the referrer.

HANDOFF 2 — Referrer to receiving team by email or e-RS. Information moves. Responsibility is assumed to move.

HANDOFF 3 — Mailbox to PAS. Admin re-keys. Information moves, nobody confirms receipt.

HANDOFF 4 — Triager to booking. One conversion carries patient advice and admin-only instructions. Responsibility moves inside the trust.

HANDOFF 5 — Booking to consultant and back for borderline cases. Ownership lives in email threads.

HANDOFF 6 — RTT team back to the referrer: does the patient still need treatment?

HANDOFF 7 — Clinician to admin by email or form: "Can you change this?"

HANDOFF 8 — Trust to trust by letter or Patient Pass. Tertiary hands back by email. Biggest visibility break.

HANDOFF 9 — Admin to commissioner or panel through Blueteq.

HANDOFF 10 — Provider to GP to patient. The GP decides what the patient hears.

Visibility and audit trail

Can e-RS or anyone else see it?

1	2	3	4	5	6	7	8	9	10
e-RS for GP route only	**Email: invisible**	**PAS + X drive only**	**Visible in e-RS (A&G)**	**Split: e-RS, PAS, extract**	**PAS + RTT Tracker**	**Email + PAS**	**Invisible across trusts**	**Blueteq / CCS only**	**PAS / RTT, old UBRN live**
Audit trail: Partial	Audit trail: None	Audit trail: Weak	Audit trail: Message trail	Audit trail: Partial	Audit trail: Tracker only	Audit trail: None	Audit trail: None	Audit trail: Separate	Audit trail: Partial

Policy and clinical safety

Considerations that decide what can change

1	2	3	4	5	6	7	8	9	10
Should a new referral warn about an existing pathway?	Info governance for free-text, multi-patient email.	Lost referral is a clinical risk. Who governs group mailboxes?	Wrong-patient risk. Who owns open A&G items?	Automation must keep triage and sub-specialty nuance.	Who owns deterioration while waiting? Notification wording.	Add information without losing the patient's position.	When exactly does responsibility transfer between trusts?	RTT clock rules during funding. Appeal route.	Responsibility before and after first attendance is not confirmed policy.

Opportunities and outputs

To-be direction, capability or platform team, and tech status

1	2	3	4	5	6	7	8	9	10
TO-BE **Related-referral warning** — Track responsibility · Create & manage requests — New functionality	TO-BE **Structured request with receipt status** — Referral platform — New functionality	TO-BE **One intake for internal and e-RS referrals ("one big list")** — Referral platform — Functionality exists, no API	TO-BE **Status that shows the reason and next owner of open items** — Track responsibility — New functionality	TO-BE **Clinic and clinician directory kept current** — DOS / local directory — Functionality exists, no API	TO-BE **Clear patient status in the NHS App, not admin events** — NHS App · Track responsibility — Functionality exists, no API	TO-BE **Add to an existing referral without re-triage** — Create & manage requests — New functionality	TO-BE **Cross-trust transfer and handback status** — Track responsibility · Referral platform — New functionality	TO-BE **Funding status visible on the pathway** — Enable decisions — New functionality	TO-BE **End-to-end audit trail with write-back to PAS** — Track responsibility — Existing – reuse

What already works

- RTT Tracker App: replaced 40+ spreadsheets, shared comments, audit trail, role-based access. (P02/03)
- Protected A&G review slots with one owner stopped requests going unactioned. (P04)
- Dental to RMS to e-RS works well. Worth finding out why. (P01)
- e-RS to PAS sync of demographics and bookings on 2WW. Patient progress easy to see. (P05)
- Strong local relationships let gastro redirect patients without bouncing them back. (C1)

Two user groups in Northumbria

Group A

Would use e-RS if it had the functionality

Internal referrals (task-workflow pilot), Cumbria MSK ("would ideally arrive electronically"), mental health hospitals with no e-RS, cross-trust handbacks.

Outcome: requirements for e-RS, the referral platform and DOS.

Group B

Happy with email and relationships

Clinician-to-clinician emails behind the scenes, tertiary MDT routes, follow-up requests by email or form.

Outcome: capture their footprints another way, e.g. visibility only.

Note: in Northumbria most referrals already use e-RS. Outside e-RS is the exception, but carries most of the manual work.

Key numbers (participant estimates)

~5% non-GP referrals in surgical booking (estimate)

~10,000 gastro A&G and referrals a year, ~5% left open

200 ASIs cleared in one afternoon

6 hrs of calls on a Monday

40+ spreadsheets replaced by the RTT Tracker

Week 17 when RTT tracking starts

180 days before a patient drops off a worklist

4 months for a funding panel decision

Questions to validate

- At exactly what point does responsibility change, and is it Trust policy?
- Which system is the source of truth at each stage?
- What can the originating team see after a referral moves to another trust?
- How is an existing referral updated without creating a duplicate?
- Why does Dental to RMS to e-RS work when internal referrals don't?
- How and when is the patient told?

Systems: e-RS · OneView / EPR · Portal / NHS App · Email / mailbox · Phone · Letter · PAS · Spreadsheet / X drive · Somerset · CCS / Blueteq · ICE / results · RTT Tracker App

Routes: GP via e-RS · Internal / con to con · Advice & Guidance · 2WW / cancer · External, outside e-RS

Responsibility: CLEAR · SHARED / ASSUMED · GAP

Tech status: Existing – reuse · Functionality exists, no API · New functionality

Evidence: Anecdotal – programme · Anecdotal – user · Validated research

Cards: Handoff · Workaround · Failure · Consequence · To-be